

NORTHSHORE SPINE INSTITUTE

2200 Lakeview Parkway, Suite 300, Northbrook, IL 60062 | Tel (847) 555-0175 | Orthopedic Spine Surgery

Spine Surgery Consultation Note**Visit date:** 02/10/2026**Provider:** Anita Kessler, MD**Visit type:** Surgical consultation**Sex:** Female**Insurance:** Lakeshore Mutual Health**Member ID:** LMH-448219037**Chief Complaint**

Low back pain with left leg radiating pain and new weakness in the left foot.

HPI

Ms. Bevredeau is a 64-year-old female with low back pain and left leg radiating pain since June 2025, with no specific inciting injury. Pain radiates from the left buttock along the lateral thigh and lower leg to the top of the foot. Symptoms worsen with standing and walking and are partially relieved by sitting and forward flexion. She was referred by this office to Prairie Path Physical Therapy and completed a full course of therapy with only limited, temporary benefit. Over the past several weeks she has noticed that her left foot catches when she walks and she has tripped twice at home. Current pain is 6/10. She denies bowel or bladder dysfunction or saddle anesthesia. Prior treatment history is detailed below.

Past Medical History

Hypertension (controlled), hyperlipidemia.

Surgical history: Laparoscopic cholecystectomy (2012).

Allergies: No known drug allergies.

Social History

former smoker, quit 2019. Occasional alcohol use. Lives with spouse. Retired school administrator.

HPI (continued): Prior Treatments

Physical therapy

Completed physical therapy for 14 weeks at Prairie Path Physical Therapy (10/01/2025 to 01/08/2026; 24 visits) consisting of core stabilization, manual therapy, and a home exercise program. Discharge summary reviewed: progress plateaued with only limited, temporary improvement, and surgical consultation was recommended.

Interventional pain management

Epidural steroid injections on 11/12/2025 and 12/17/2025 provided temporary relief only, with no durable improvement in leg pain or walking tolerance.

Date	Procedure	Response
11/12/2025	Left L4-L5 transforaminal epidural steroid injection	Temporary relief only; symptoms returned to baseline within approximately 3 weeks
12/17/2025	Left L4-L5 transforaminal epidural steroid injection	Temporary relief only; symptoms returned to baseline within approximately 2 weeks

Medications tried

Medication	Dose	Response
Ibuprofen	600 mg three times daily as needed	Mild, partial relief of back pain
Acetaminophen	1,000 mg up to three times daily	Minimal relief
Gabapentin	300 mg three times daily	Modest reduction in leg pain; no change in function
Cyclobenzaprine	10 mg at bedtime	Helps with sleep; no effect on leg pain

Other measures

Activity modification, avoidance of prolonged standing, heat, and continuation of the home exercise program from physical therapy.

Review of Systems

Negative for fever, chills, unexplained weight loss, night sweats, and bowel or bladder changes. Positive for low back pain, left leg pain, and left foot weakness as noted in the HPI. All other systems reviewed and negative.

Physical Exam

Vitals: BP 132/80, HR 74, BMI 27.4.

General: Well-appearing, in mild discomfort when standing.

Gait: Mildly antalgic with subtle left foot slap; difficulty with heel walking on the left.

Lumbar spine: Tenderness over the lower lumbar paraspinals. Flexion and extension limited by pain; extension reproduces left buttock pain. Straight leg raise positive on the left, negative on the right.

Motor: Progressive left foot dorsiflexion weakness, 4/5. All other lower extremity muscle groups 5/5.

Muscle group	Right	Left
Hip flexion	5/5	5/5
Knee extension	5/5	5/5
Ankle dorsiflexion	5/5	4/5
Ankle plantarflexion	5/5	5/5

Reflexes: Patellar and Achilles 2+ and symmetric. No clonus; Babinski negative bilaterally.

Sensation: Mildly diminished light touch over the left dorsal foot.

Imaging Review

MRI of the lumbar spine dated 01/20/2026 (Lakeview Imaging Center) was personally reviewed and demonstrates **Grade II spondylolisthesis at L4-L5** with bilateral facet arthropathy and moderate left foraminal stenosis. L5-S1 shows mild disc desiccation without stenosis or listhesis. L1-L2 through L3-L4 are unremarkable. Imaging findings at L4-L5 correlate with her left leg symptoms and examination findings.

Assessment

1. Degenerative spondylolisthesis, L4-L5 (ICD-10 M43.16).
2. Left lumbar radiculopathy (ICD-10 M54.16) with progressive left foot dorsiflexion weakness.
3. Failure of conservative care, including physical therapy, epidural steroid injections, and oral medications.

Plan

We discussed operative and non-operative options in detail. Given her persistent radicular pain, progressive motor weakness, and failure of conservative care, I have recommended surgical treatment and she wishes to proceed.

Planned procedure: L4-L5 posterior lumbar interbody fusion (PLIF), single level; CPT 22630

Surgery date: Scheduled for 03/21/2026

Surgeon: Anita Kessler, MD

- Risks, benefits, and alternatives reviewed, including infection, bleeding, nerve injury, dural tear, nonunion, adjacent segment disease, and the possibility of incomplete symptom relief. Questions answered.
- Pre-operative medical clearance with primary care physician and routine pre-operative laboratory studies.
- Patient denies current nicotine use; counseled to remain nicotine-free before and after surgery.
- Continue home exercise program and current medications until surgery. Hold ibuprofen 7 days before surgery.
- Return precautions given for worsening weakness, new numbness, or bowel or bladder changes.

Electronically signed by: Anita Kessler, MD, Orthopedic Spine Surgery, Northshore Spine Institute

Signed: 02/10/2026

Addendum (02/12/2026)

Prior authorization submitted 02/12/2026 to Lakeshore Mutual Health for the L4-L5 posterior lumbar interbody fusion listed above (Prior Authorization #PA-2026-0219884). Clinical notes, physical therapy discharge summary, and MRI report included with the request.

Addendum electronically signed by: Anita Kessler, MD, 02/12/2026